

Mr

2-2548

CD : PT EVAL Questionnaire

Date: 8/27

Time:

Name: Mr. Elvel, Dorey

88/710

What is most important for patient today? Walking / getting better / able to get out of bed/

Medical dx: (Hx), DM2

Physical therapy DX: slr full, weakness

Subjective: patient feeling weak, ---

Vitals: BP: 124/60 HR: 60 Temp: 97.2 Resp: 17

Prior level of functioning:

Bed mobility: Independent / Needs assistance / Dependent

Transfers: Independent / Needs assistance / Dependent

Gait: Independent / Needs assistance / Dependent

Stairs: Independent / Needs assistance / Dependent

DME: Cane / Walker / wc

Eyes: Glasses: Y/N N Reading/Distance Glaucoma, Cataracts, Macular Degeneration

Hearing: Impaired/ deaf/ Hearing aids: R/L Good

Pain: Location (B) Ankle : 4/10

Type:

Score:

4 / 10

Oriented to person, place, time, situation / alert / disoriented / confused /

Forgetful / PEERL / Seizures / Tremors

Neurological deficit: none / (B) Leg: tingling, neuropathy

Environmental problem: none /

Cognitive problem: dementia / behaviour problem / none /

Current Level of functioning:

Goals:

ROM: B/L UE WFL B/L LE: WFL

Strength B/L UE 3/5 B/L LE: 3/5 → 3+/5

Rolling: inde/VC/SBA/CGA/Min A/Mod A/MaxA/Dep

Supine to sit: inde/VC/SBA/CGA/Min A/Mod A/MaxA/Dep

Scoot: indeVC/SBA/CGA/Min A/Mod A/MaxA/Dep

Transfers: inde/VC/SBA/CGA/Min A/Mod A/MaxA/Dep

→ indep
→ indep

Standing balance: Poor/Poor+ Fair/Fair+/Normal → Fair

Gait: inde/ mod inde/SBA/CGA/Min A/Mod A/Max A/Unable, Feet: 15/20/30/40 → 150A (Fair)

Gait problems: unsteady, shuffling, limping, antalgic, parkinson's gait, Ataxic gait, Hemiplegic gait, narrow base, dragging both legs and scraping the toes, Nueropathic gait (foot drop), Myopathic gait (hip imbalance), Decreased step length, decrease Cadence, decreased floor clearance, wide Base

Wheelchair: inde/VC/SBA/CGA/Min A/Mod A/MaxA/Dep/NA

Stair/steps: inde/VC/SBA/CGA/Min A/Mod A/MaxA/Dep/NA

Weight bearing status: FWB

Device used: FWW

DME Available: 2WW/4WW/Cane/Crutches/Wheelchair/Hospital Bed/ Commode/showerchair/Hoyerlift

DME needs:

DC from: Hospital due to: S/P Fall dc from transfers to SNF: 2 weeks DC date: 8/21

Lives assistance: Alone: Home with: 2 hrs Facility:

Caregiver available: Y/N hours available: 2h

Oxygen: Y/N How many Litres: Via: Nasal Canula. How many hours: PRN/6/12/24/Nigh

PT Frequency: 1wk1, 2wk2, 1wk1

Rehab : poor/fair/good/ guarded

Prognosis: poor/fair/good/ guarded